

TREMFYA (guselkumab)

Line(s) of Business:
HMO; PPO; QUEST

Original Effective Date:
10/01/2020
Current Effective Date:
04/01/2025

POLICY

A. INDICATIONS

The indications below including FDA-approved indications and compendial uses are considered a covered benefit provided that all the approval criteria are met and the member has no exclusions to the prescribed therapy.

FDA-Approved Indications

- Treatment of adult patients with moderate-to-severe plaque psoriasis (PsO) who are candidates for systemic therapy or phototherapy
- Treatment of adult patients with active psoriatic arthritis (PsA)
- Treatment of moderately to severely active ulcerative colitis (UC) in adults

B. PRESCRIBER SPECIALTIES

This medication must be prescribed by or in consultation with one of the following:

1. Dermatologist
 - a. Plaque psoriasis or psoriatic arthritis
2. Rheumatologist
 - a. Psoriatic arthritis
3. Gastroenterologist
 - a. Ulcerative colitis

C. CRITERIA FOR INITIAL APPROVAL

1. Plaque psoriasis (PsO)

- a. Authorization of 12 months may be granted for adult members who have previously received a biologic or targeted synthetic drug (e.g., Sotyktu, Otezla) indicated for the treatment of moderate to severe plaque psoriasis.
- b. Authorization of 12 months may be granted for adult members for treatment of moderate to severe plaque psoriasis when any of the following criteria is met:
 - i. Crucial body areas (e.g., hands, feet, face, neck, scalp, genitals/groin, intertriginous areas) are affected
 - ii. At least 10% of body surface area (BSA) is affected
 - iii. At least 3% of body surface area (BSA) is affected and the member meets any of the following criteria:

- a. Member has had an inadequate response or intolerance to either phototherapy (e.g., UVB, PUVA) or pharmacologic treatment with methotrexate, cyclosporine or acitretin.
- b. Member has a clinical reason to avoid pharmacologic treatment with methotrexate, cyclosporine and acitretin (see Appendix).

2. Psoriatic arthritis (PsA)

- a. Authorization of 12 months may be granted for adult members who have previously received a biologic or targeted synthetic drug (e.g., Rinvoq, Otezla) indicated for active psoriatic arthritis.
- b. Authorization of 12 months may be granted for adult members for treatment of active psoriatic arthritis when either of the following criteria is met:
 - i. Member has mild to moderate disease and meets one of the following criteria:
 - 1. Member has had an inadequate response to methotrexate, leflunomide, or another conventional synthetic drug (e.g., sulfasalazine) administered at an adequate dose and duration.
 - 2. Member has an intolerance or contraindication to methotrexate or leflunomide (see Appendix), or another conventional synthetic drug (e.g., sulfasalazine).
 - 3. Member has enthesitis.
 - ii. Member has severe disease.

3. Ulcerative colitis (UC)

Authorization of 12 months may be granted for adult members for treatment of moderately to severely active ulcerative colitis.

D. CONTINUATION OF THERAPY

1. No previous authorization/precertification:

All members (including new members and members currently receiving treatment without prior authorization) must meet criteria for initial approval in section C.

2. Reauthorization:

A. Plaque psoriasis (PsO)

Authorization of 12 months may be granted for adult members who were previously authorized by HMSA/HMSA's Pharmacy Benefit Manager and achieve or maintain a positive clinical response from Tremfya therapy as evidenced by low disease activity or improvement in signs and symptoms of the condition when either of the following is met:

- 1. Reduction in body surface area (BSA) affected from baseline
- 2. Improvement in signs and symptoms from baseline (e.g., itching, redness, flaking, scaling, burning, cracking, pain)

B. Psoriatic arthritis (PsA)

Authorization of 12 months may be granted for adult members who were previously authorized by HMSA/HMSA's Pharmacy Benefit Manager and achieve or maintain a positive clinical response from Tremfya therapy as evidenced by low disease activity or improvement in signs and symptoms of the condition when there is improvement in any of the following from baseline:

- 1. Number of swollen joints
- 2. Number of tender joints
- 3. Dactylitis

4. Enthesitis
5. Skin and/or nail involvement
6. Functional status
7. C-reactive protein (CRP)

C. Ulcerative colitis

1. Authorization of 12 months may be granted for adult members (including new members) who were previously authorized by HMSA/CVS and who achieve or maintain remission.
2. Authorization of 12 months may be granted for adult members who were previously authorized by HMSA/HMSA's Pharmacy Benefit Manager and who achieve or maintain a positive clinical response as evidenced by low disease activity or improvement in signs and symptoms of the condition when there is improvement in any of the following from baseline:
 - a. Stool frequency
 - b. Rectal bleeding
 - c. Urgency of defecation
 - d. C-reactive protein (CRP)
 - e. Fecal calprotectin (FC)
 - f. Appearance of the mucosa on endoscopy, computed tomography enterography (CTE), magnetic resonance enterography (MRE), or intestinal ultrasound
 - g. Improvement on a disease activity scoring tool (e.g., Ulcerative Colitis Endoscopic Index of Severity [UCEIS], Mayo score)

E. OTHER

For all indications: Member has had a documented negative tuberculosis (TB) test (which can include a tuberculosis skin test [TST] or an interferon-release assay [IGRA])* within 6 months of initiating therapy for persons who are naïve to biologic drugs or targeted synthetic drugs associated with an increased risk of TB.

* If the screening testing for TB is positive, there must be further testing to confirm there is no active disease (e.g., chest x-ray). Do not administer the requested medication to members with active TB infection. If there is latent disease, TB treatment must be started before initiation of the requested medication.

For all indications: Member cannot use the requested medication concomitantly with any other biologic drug or targeted synthetic drug for the same indication.

F. APPENDIX

Examples of Clinical Reasons to Avoid Pharmacologic Treatment with Methotrexate, Cyclosporine, Acitretin or Leflunomide

1. Clinical diagnosis of alcohol use disorder, alcoholic liver disease or other chronic liver disease
2. Drug interaction
3. Risk of treatment-related toxicity
4. Pregnancy or currently planning pregnancy
5. Breastfeeding
6. Significant comorbidity prohibits use of systemic agents (e.g., liver or kidney disease, blood dyscrasias, uncontrolled hypertension)
7. Hypersensitivity
8. History of intolerance or adverse event

G. DOSAGE AND ADMINISTRATION

Approvals may be subject to dosing limits in accordance with FDA-approved labeling, accepted compendia, and/or evidence-based practice guidelines.

H. ADMINISTRATIVE GUIDELINES

Precertification is required. Please refer to the [HMSA medical specialty drug policy web site](#) for the fax form.

I. IMPORTANT REMINDER

The purpose of this Medical Policy is to provide a guide to coverage. This Medical Policy is not intended to dictate to providers how to practice medicine. Nothing in this Medical Policy is intended to discourage or prohibit providing other medical advice or treatment deemed appropriate by the treating physician.

Benefit determinations are subject to applicable member contract language. To the extent there are any conflicts between these guidelines and the contract language, the contract language will control.

This Medical Policy has been developed through consideration of the medical necessity criteria under Hawaii's Patients' Bill of Rights and Responsibilities Act (Hawaii Revised Statutes §432E-1.4), generally accepted standards of medical practice and review of medical literature and government approval status. HMSA has determined that services not covered under this Medical Policy will not be medically necessary under Hawaii law in most cases. If a treating physician disagrees with HMSA/ HMSA's Pharmacy Benefit Manager's determination as to medical necessity in a given case, the physician may request that HMSA reconsider the application of the medical necessity criteria to the case at issue in light of any supporting documentation.

J. REFERENCES

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11. Menter A, Gelfand JM, Connor C, et al. Joint American Academy of Dermatology-National Psoriasis Foundation guidelines of care for the management of psoriasis with systemic nonbiologic therapies. *J Am Acad Dermatol*. 2020;82(6):1445-1486.
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Document History

10/01/2020	Original effective date
5/14/2021	Removed Medicare LOB per SAD Exclusion List
05/2021	Updated continuation of therapy to align with HMSA's Pharmacy Benefit Manager standard
06/2021	Annual review
01/01/2022	Revision effective date
04/2022	Updated TB test requirements
06/2022	Annual review
09/01/2022	Revision effective date – Annual review and off-cycle update
07/2023	Annual review, removed HMSA's Pharmacy Benefit Manager co-branding
02/01/2024	Revision effective date
07/2024	Annual review
04/01/2025	Revision effective date